



Smart Report

Patient Name : NITA SINGH
Age/Gender : 58 Years/Female
UHID : DPPU.0000002063
Location : --

Sample Collection : 2024-02-13T09:58:41
Date
Mobile Number : --
Blood Group : --

Lab Report Summary

Heart

- 6 Parameters
- 1 Parameters
- 2 Parameters

Blood

- 15 Parameters
- 3 Parameters
- 3 Parameters

kidney

- 20 Parameters
- 1 Parameters

Glands & Endocrine System

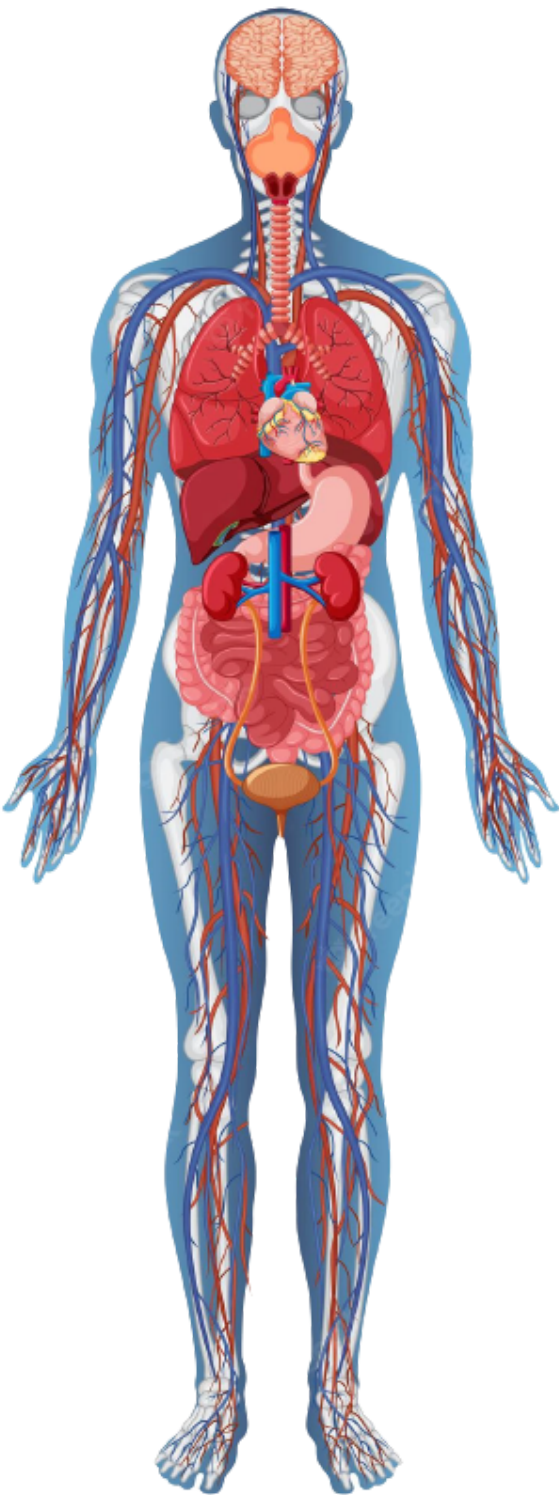
- 3 Parameters
- 1 Parameters
- 1 Parameters

Stomach & Liver/Digestive System

- 6 Parameters
- 2 Parameters

General

- 1 Parameters
- 1 Parameters





Patient Info

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System Wise Lab Tests

Heart

- TOTAL CHOLESTEROL
- HDL CHOLESTEROL
- LDL CHOLESTEROL
- CHOL / HDL RATIO
- RHEUMATOID FACTOR
- TRIGLYCERIDES
- NON-HDL CHOLESTEROL
- VLDL CHOLESTEROL
- CA -125

Blood

- HAEMOGLOBIN
- RBC COUNT
- MCH
- R.D.W
- NEUTROPHILS
- EOSINOPHILS
- BASOPHILS
- LYMPHOCYTES
- MONOCYTES
- PLATELET COUNT
- ERYTHROCYTE SEDIMENTATION RATE (ESR)
- PCV
- MCV
- MCHC
- TOTAL LEUCOCYTE COUNT (TLC)
- LYMPHOCYTES
- MONOCYTES
- NEUTROPHILS
- EOSINOPHILS
- BASOPHILS
- IRON

kidney

- CREATININE
- BLOOD UREA NITROGEN
- CALCIUM
- SODIUM
- CHLORIDE
- SP. GRAVITY
- GLUCOSE
- URINE KETONES (RANDOM)
- NITRITE
- RBC
- CRYSTALS
- UREA
- URIC ACID
- PHOSPHORUS, INORGANIC
- POTASSIUM
- pH
- URINE PROTEIN
- URINE BILIRUBIN
- BLOOD
- LEUCOCYTE ESTERASE
- CASTS

Glands & Endocrine System

- GLUCOSE, FASTING
- TRI-IODOTHYRONINE (T3, TOTAL)
- THYROID STIMULATING HORMONE (TSH)
- HBA1C, GLYCATED HEMOGLOBIN
- THYROXINE (T4, TOTAL)

Stomach & Liver/Digestive System

- BILIRUBIN, TOTAL
- BILIRUBIN (INDIRECT)
- PROTEIN, TOTAL
- GLOBULIN
- BILIRUBIN CONJUGATED (DIRECT)
- ALKALINE PHOSPHATASE
- ALBUMIN
- A/G RATIO

General

- VITAMIN B12
- VITAMIN D (25 - OH VITAMIN D)



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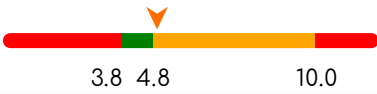


COMPLETE BLOOD COUNT (CBC)

Haemogram/Complete Blood Count is a test panel that gives information about the cells in a your blood. Abnormally high or low counts may indicate the presence of many forms of disease

RBC COUNT

4.94 Million/cu.mm



3.8 - 4.8 Million/cu.mm

Info- Red Blood Count is the amount of Red Blood Cells per unit of your blood

Inference- Your RBC Counts are Slightly Higher than Normal

TOTAL LEUCOCYTE COUNT (TLC)

7920 cells/cu.mm

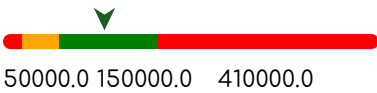


4000 - 10000 cells/cu.mm

Info- Leucocytes also known as white blood cells (WBCs) are colorless cells that circulate in your blood and lymph that have major contribution towards your body's Immune system

PLATELET COUNT

272000 cells/cu.mm

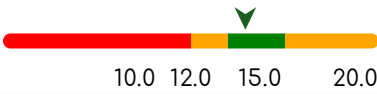


150000 - 410000 cells/cu.mm

Info- Platelets, or thrombocytes, are small, colorless cell fragments in our blood that form clots and stop or prevent bleeding.

HAEMOGLOBIN

12.9 g/dL



12 - 15 g/dL

Info- Hemoglobin is a protein in your red blood cells that carries oxygen to your body's organs and tissues and transports carbon dioxide from your organs and tissues back to your lungs. Lower Hemoglobin levels are defined as Anemia

Blood Indices

PCV	38.6 %	36 - 46 %	Normal
MCH	26.1 pg	27 - 32 pg	Normal
MCHC	33.4 g/dL	31.5 - 34.5 g/dL	Normal
MCV	78.2 fL	83 - 101 fL	Normal
R.D.W	14.6 %	11.6 - 14 %	Normal

Differential Leucocyte count

EOSINOPHILS	1.1 %	1 - 6 %	Normal
BASOPHILS	0.2 %	0 - 2 %	Normal
MONOCYTES	6.8 %	2 - 10 %	Normal
NEUTROPHILS	61.6 %	40 - 80 %	Normal
LYMPHOCYTES	30.3 %	20 - 40 %	Normal

NEUTROPHILS 4878.72 Cells/cu.mm	LYMPHOCYTES 2399.76 Cells/cu.mm	CORRECTED TLC 7920 Cells/cu.mm	EOSINOPHILS 87.12 Cells/cu.mm
MONOCYTES 538.56 Cells/cu.mm	BASOPHILS 15.84 Cells/cu.mm	Neutrophil lymphocyte ratio (NLR) 2.03	



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IRON STUDIES (IRON + TIBC)

A serum iron test assess the amount of iron in your blood. A total iron-binding capacity (TIBC) test measures the blood's ability to attach itself to iron and transport it around your body. This test is done to check for medical conditions that cause abnormal iron levels.

IRON

59.2 $\mu\text{g/dL}$

40.060.0180.0

60 - 180 $\mu\text{g/dL}$

- i

Info- A serum iron test assess the amount of iron in your blood. Iron Deficiency is most commonly known to cause Anemia/Low Hemoglobin
- 👁

Inference- Your Serum Iron Levels are Lower than Normal

TOTAL IRON BINDING CAPACITY (TIBC)

388 $\mu\text{g/dL}$

261.0462.0

261 - 462 $\mu\text{g/dL}$

- i

Info- A total iron-binding capacity TIBC test measures your blood's ability to attach itself to iron and transport it around your body. This test is done to check for medical conditions that cause abnormal iron levels.

UNSATURATED IRON BINDING CAPACITY (UIBC)

328.5 IU/mL

155.0355.0

155 - 355 IU/mL

% OF SATURATION

15.26 %

14.050.0

14 - 50 %



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VITAMIN B12

Vitamin B12 is a vitamin that is required for DNA Synthesis in your body in turn required to keep your Nerves and Blood Cells Healthy

VITAMIN B12

471 pg/mL



107.2 - 653.3 pg/mL

 Info- Low Vitamin B12 Levels can cause Anemia and also can affect your brain functions



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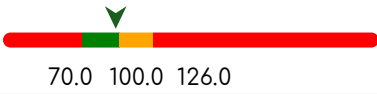


GLUCOSE, FASTING

Fasting Glucose is the measurement of levels of blood glucose after fasting/not consuming food for 10-12 hours

GLUCOSE, FASTING

96 mg/dL



70 - 100 mg/dL



Info- Fasting Glucose when abnormal is divided into ranges - Prediabetic and Diabetic. Prediabetic Ranges can usually be managed using lifestyle changes



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HBA1C, GLYCATED HEMOGLOBIN

The HBA1C test measures the average blood sugar level over the past 3 months. This test helps in evaluating the long-term control of blood glucose concentrations in patients with diabetes

ESTIMATED AVERAGE GLUCOSE (eAG)

123 mg/dL

Info- Estimated Average Glucose EAg are h type concentrations white helps cells which pati past majportion concentrations past immues response

HBA1C, GLYCATED HEMOGLOBIN

5.9 %



0 - 5.7 %

Info- HBA1C indicates the range of glucose that was in your blood for the past few months

Inference- Your Sugar Levels are in PreDiabetic Range



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LIVER FUNCTION TEST (LFT)

Liver function tests are blood tests used to help diagnose and monitor liver disease or damage by measuring the levels of certain enzymes and proteins made by the liver. LFT is done to measure the levels of - alanine aminotransferase (ALT) alkaline phosphatase (ALP), aspartate aminotransferase (AST), gamma-glutamyl transferase (GGT), bilirubin, albumin, total protein.

ALANINE AMINOTRANSFERASE (ALT/SGPT)

17 U/L

Info- ALT/SGPT is an enzyme present in your liver cells and is released into your blood in an excess amount if there is any liver damage

ASPARTATE AMINOTRANSFERASE (AST/SGOT)

22 U/L

Info- AST/SGOT is an enzyme that is similar to SGPT and in the same way its levels in your blood increase in case of any liver damage

BILIRUBIN, TOTAL

0.68 mg/dL

0.3 0.3 1.2

0.3 - 1.2 mg/dL

Info- Bilirubin is a substance that is formed when your body breaks down the old red blood cells.

BILIRUBIN CONJUGATED (DIRECT)

0.11 mg/dL

0.0 0.2

0 - 0.19 mg/dL

Info- Conjugated Bilirubin is the bilirubin that is formed after its processed by the liver

BILIRUBIN (INDIRECT)

0.57 mg/dL

0.0 1.1

0 - 1.1 mg/dL

Info- Indirect/Unconjugated Bilirubin is the bilirubin that circulates in the blood before its further processed by the liver

ALKALINE PHOSPHATASE

67 U/L

120.0

30 - 120 U/L

Info- Alkaline Phosphatase is an enzyme that's present in your body and its levels are elevated in various conditions related to the liver

PROTEIN, TOTAL

8.31 g/dL

6.6 8.3

6.6 - 8.3 g/dL

Info- Total Protein test gives the combined amount of Albumin and Globulin in your blood

Inference- Your Total Protein Levels are Higher than Normal

ALBUMIN

4.4 g/dL

3.5 5.2

3.5 - 5.2 g/dL

Info- Albumin is a type of protein that is generated by your liver

GLOBULIN

3.91 g/dL

2.0 3.5

2 - 3.5 g/dL

Info- Globulin is the other type of protein like albumin made by your liver

Inference- Your Globulin Levels are Higher than Normal

A/G RATIO

1.13

0.9 0.9 2.0

0.9 - 2

Info- The Ratio of the Albumin to Globulin levels in your body is measured as there are certain conditions that can alter the relative amounts of these two proteins



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LIPID PROFILE

The different kinds of cholesterol and other fats in the blood are together called lipids. A Lipid profile gives results for four different types which is Total Cholesterol, LDL - Bad cholesterol HDL - Good Cholesterol and Triglycerides - Common type of fat in the body

TOTAL CHOLESTEROL

213 mg/dL

0.0200.0240.0

0 - 199.99 mg/dL

Info- Total Amount of all types of Cholestrol in your body

Inference- Your Total Cholesterol values are Borderline High

TRIGLYCERIDES

133 mg/dL

0.0150.0200.0

0 - 149.99 mg/dL

Info- Triglycerides are a type of fats present in your body

HDL CHOLESTEROL

50 mg/dL

40.060.0

40 - 60 mg/dL

Info- HDL Cholesterol is also referred to as the "Good Cholesterol"

NON-HDL CHOLESTEROL

163 mg/dL

0.0130.0

0 - 130 mg/dL

Info- As HDL Cholestrol is defined as the good cholestrol the other forms of cholestrol that are harmful to your body are included in this parameter

Inference- Values are Higher than Normal

LDL CHOLESTEROL

136.4 mg/dL

0.0100.0

0 - 100 mg/dL

Info- LDL Cholesterol is also referred to as the "Bad Cholesterol"

Inference- Your LDL Levels are on the higher end

VLDL CHOLESTEROL

26.6 mg/dL

5.030.0

5 - 30 mg/dL

Info- VLDL Cholesterol i.e Very-low-density protein produced in the liver

CHOL / HDL RATIO

4.26

0.05.0

0 - 4.97

Info- Cholesterol - HDL Ratio depicts the Good vs bad cholesterol ratio in your body. Higher the value, higher the risk of complications



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THYROID PROFILE (TOTAL T3, TOTAL T4, TSH)

Thyroid profile test is done to assess the proper functioning of thyroid gland. Thyroid hormones regulate many aspects of the body's metabolism, including temperature, weight, and energy.

TRI-iodothyronine (T3, TOTAL)

1.24 ng/mL

0.87

0.87 - 1.78 ng/mL

Info- Tri-iodothyronine is a hormone that plays vital role in growth and development of the body, maintenance of vital parameters like temperature and heart rate etc

THYROXINE (T4, TOTAL)

12.23 µg/dL

5.514.3

5.48 - 14.28 µg/dL

Info- Thyroxine is a hormone that plays vital role in digestion, brain development, bone maintenance etc

THYROID STIMULATING HORMONE (TSH)

6.047 µIU/mL

0.385.3

0.38 - 5.33 µIU/mL

Info- Thyroid Stimulating Hormone as the name suggests stimulates the Thyroid gland to produce thyroxine

Inference- Your TSH values are higher than normal



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VITAMIN D - 25 HYDROXY (D2+D3)

Vitamin D is a nutrient that is essential to keep your bones and teeth healthy. Vitamin D test is done to assess levels of Vitamin D in the body.

VITAMIN D (25 - OH VITAMIN D)

26.9 ng/mL



30 - 100 ng/mL

 Info- Deficiency of Vitamin D results in Weaker Bones in your body

 Inference- Your Vitamin D Levels Signify you have Vitamin D Insufficiency



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ERYTHROCYTE SEDIMENTATION RATE (ESR)

Erythrocyte Sedimentation Rate is the rate at which Red blood cells in your body can sediment to the bottom of the blood sample

ERYTHROCYTE SEDIMENTATION RATE (ESR)

37 mm at the end of 1 hour



0 - 20 mm at the end of 1 hour

Info- Erythrocyte Sedimentation Rate is increased when there is inflammation in the body

Inference- Your ESR Values are Higher than Normal



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COMPLETE URINE EXAMINATION

Complete Urine examination is urinalysis done on your urine to detect and manage a wide range of disorders, such as urinary tract infections, kidney or liver disease and diabetes. The parameters that are checked in urine are Acidity, Concentration, Protein, Sugar, Ketones, Nitrites, Bilirubin. Levels of WBs, RBCs, Bacteria, Yeasts, Casts and Crystals are assessed.

COLOUR PALE YELLOW	TRANSPARENCY CLEAR	URINE PROTEIN NEGATIVE	GLUCOSE NEGATIVE
URINE BILIRUBIN NEGATIVE	URINE KETONES (RANDOM) NEGATIVE	UROBILINOGEN NORMAL	BLOOD NEGATIVE
NITRITE NEGATIVE	LEUCOCYTE ESTERASE NEGATIVE	PUS CELLS 3-4 /hpf	EPITHELIAL CELLS 1-2 /hpf
RBC NIL /hpf	CASTS NIL	CRYSTALS ABSENT	

pH	6		5 - 7.5
SP. GRAVITY	1.02		1.002 - 1.03



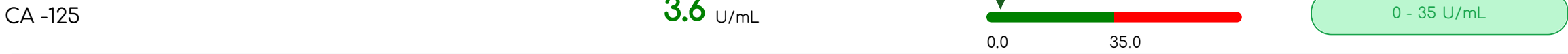
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CA -125

A CA 125 test measures the amount of the protein CA 125 (cancer antigen 125) in your blood.



i Info- A CA 125 test may be used to monitor certain cancers during and after treatment.



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RHEUMATOID FACTOR (RA) QUANTITATIVE

Rheumatoid factors are proteins produced by the immune system, and a RF test is done to measure the amount of Rheumatoid Factor present in the blood.

RHEUMATOID FACTOR

3.1 IU/mL



0 - 14 IU/mL

i Info- An RF test is used to help diagnose rheumatoid arthritis or other autoimmune disorders



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Recommendations



Test Recommendations

Follow Up Recommendation

- FT3, FT4
- Serum Calcium
- CRP

Other Recommendations

- Anti TPO antibodies
- PTH
- Allergy Panel



Speciality Recommendations

Follow Up Recommendation

- Endocrinologist
- General Medicine

Other Recommendations

- Dermatologist

Note: Follow up tests are recommended based on laboratory report analysis, please reach to your doctor to seek advice regarding the following additional tests.



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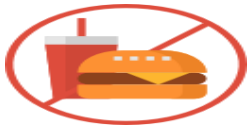
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Personal Recommendations



Diet recommendations

Do's

- Eat fruits, such as bananas, citrus fruits and melons which are rich sources of iron
- Eat iron-rich foods with foods that contain beta carotene, such as apricots, red peppers, and beets, to improve absorption
- Eat iron-rich foods with vitamin C-rich foods, such as oranges, tomatoes, or strawberries, to improve absorption.
- Include fistful of almonds & walnuts per day, 1-2 teaspoons of flax seeds per day
- Include Iron rich diet like green leafy vegetables like Spinach, Legumes like Kidney beans etc
- Add dietary fibre in your diet (i.e. Oats, Chickpeas, Kidney Beans, Lentils)
- Add healthy fats (i.e. Avacados, Almonds, Eggs, Fatty fish) in your diet.
- Add nuts (i.e. Almonds, Walnuts) and seeds (i.e. Chia, Flax, Pumpkin) in your diet.
- Eat whole grains - i.e. Whole wheat flour in place of refined flour

Dont's

- Eat iron-rich foods with foods or beverages that block iron absorption. These include coffee or tea etc
- Overcook your food.
- Consume simple carbs (i.e. White Bread, Refined Flour (Maida), White Rice, Sweetened Cereals)
- Consume sugar-sweetened beverages (i.e. Soft Drinks, Carbonated Drinks, Sweet Lassi)
- Consumesugary fruits in large quantity (i.e. Watermelons, Riped Mango, Pineapples, Dates)



Physical Activity

Do's

- Moderate walking for 30 minutes if health permits.
- Exercise more and keep yourself active. Start with walking after lunch for 20-30 minutes.
- Keep yourself active through out the day.Specially in outdoor activities



Lifestyle Recommendations

Do's

- Do Meditation for 10-15 minutes per day.
- Ensure adequate rest such as such as relaxing at home or on vacation etc.
- Ensure proper sleep for at least 7 hrs.
- Distribute your carb intake through out the day.
- Lose weight if you are overweight. Check your BMI - Normal range 18.5—24.9 BMI over 25 is considered as overweight.
- Track what you eat.
- Motinor what you eat - Count calories and total fat intake.
- Replace your bad fat (i.e Butter, Margarine) with good fat (i.e. Vegetable Oil, Peanut Butter) in general.
- Try fasting in supervision
- Your total fat intake should be less than 25% of what you eat.
- Get adequate exposure to sunlight, 10 min everyday. Careful if your skin is sensitive, avoid too much exposure.
- Keep yourself active through out the day.Specially in outdoor activities

Dont's

- Consume Tobacco & alcohol
- Consume alcohol
- Smoke or take tobacco in any form

Disclaimer :

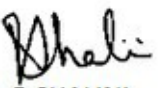
- This report is for educating patients (especially in diet & lifestyle).
- This report is not intended to replace your clinicians, we recommend to contact your clinicians for interpretation of result.
- If you are pregnant, some of the recommendations in the Smart Report may not directly apply to you. Please consult your doctor.
- The analyzed information in the Smart Report is not ideal for individuals less than 15 years of age. Please make sure you consult your doctor before further actions.
- Please be careful of any food allergies or intolerances that you are sensitive to.

Patient Name	: Mrs.NITA SINGH	Collected	: 13/Feb/2024 09:58AM
Age/Gender	: 58 Y 9 M 10 D /F	Received	: 13/Feb/2024 01:21PM
UHID/MR No	: DPPU.0000002063	Reported	: 13/Feb/2024 02:35PM
Visit ID	: DPPUOPV4758	Status	: Final Report
Ref Doctor	: Dr.SELF	Client Name	: PCC UPPERPALLY
IP/OP NO	:	Patient location	: ,Hyderabad

DEPARTMENT OF HAEMATOLOGY

XPERT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
COMPLETE BLOOD COUNT (CBC) , WHOLE BLOOD EDTA				
HAEMOGLOBIN	12.9	g/dL	12-15	Spectrophotometer
PCV	38.60	%	36-46	Electronic pulse & Calculation
RBC COUNT	4.94	Million/cu.mm	3.8-4.8	Electrical Impedance
MCV	78.2	fL	83-101	Calculated
MCH	26.1	pg	27-32	Calculated
MCHC	33.4	g/dL	31.5-34.5	Calculated
R.D.W	14.6	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	7,920	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	61.6	%	40-80	Electrical Impedance
LYMPHOCYTES	30.3	%	20-40	Electrical Impedance
EOSINOPHILS	1.1	%	1-6	Electrical Impedance
MONOCYTES	6.8	%	2-10	Electrical Impedance
BASOPHILS	0.2	%	0-2	Electrical Impedance
CORRECTED TLC	7,920	Cells/cu.mm		Calculated
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	4878.72	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	2399.76	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	87.12	Cells/cu.mm	20-500	Calculated
MONOCYTES	538.56	Cells/cu.mm	200-1000	Calculated
BASOPHILS	15.84	Cells/cu.mm	0-100	Calculated
Neutrophil lymphocyte ratio (NLR)	2.03		0.78- 3.53	Calculated
PLATELET COUNT	272000	cells/cu.mm	150000-410000	Electrical impedance


Dr.R.SHALINI
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:HA06481859

This test has been performed at Apollo Health & Lifestyle Ltd, Global Reference Laboratory,Hyderabad



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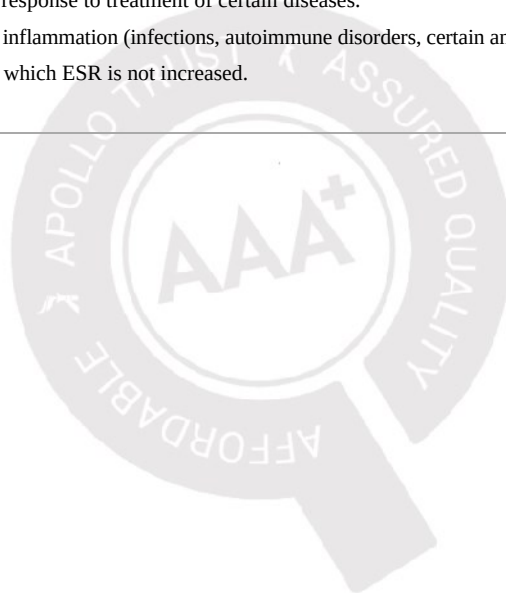
DEPARTMENT OF HAEMATOLOGY

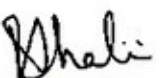
XPERT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
ERYTHROCYTE SEDIMENTATION RATE (ESR) , WHOLE BLOOD EDTA	37	mm at the end of 1 hour	0-20	Modified Westergren

Comment:

1. ESR is an acute phase reactant which indicates presence and severity of an inflammatory process. ESR is not diagnostic of any specific disease. ESR is a screening test with low sensitivity. CRP has better sensitivity than ESR.
2. ESR Test is used to monitor the course or response to treatment of certain diseases.
3. ESR is an indirect measure of the extent of inflammation (infections, autoimmune disorders, certain anemias & malignancies).
4. There are many diseases such as typhoid in which ESR is not increased.




Dr.R.SHALINI
M.B.B.S,M.D(Pathology)
Consultant Pathologist

SIN No:HA06481859

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Apollo Health and Lifestyle Limited

(CIN - U85110TG2000PLC115819)

Corporate Office: 7-1-617/A, 7th Floor, Imperial Towers, Ameerpet, Hyderabad-500016, Telangana

Ph No: 040-4904 7777 | www.apollohl.com | Email ID:enquiry@apollohl.com

Patient Name	: Mrs.NITA SINGH	Collected	: 13/Feb/2024 09:58AM
Age/Gender	: 58 Y 9 M 10 D /F	Received	: 13/Feb/2024 01:09PM
UHID/MR No	: DPPU.0000002063	Reported	: 13/Feb/2024 01:38PM
Visit ID	: DPPUOPV4758	Status	: Final Report
Ref Doctor	: Dr.SELF	Client Name	: PCC UPPERPALLY
IP/OP NO	:	Patient location	: ,Hyderabad

DEPARTMENT OF BIOCHEMISTRY

XPERT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
GLUCOSE, FASTING , NAF PLASMA	96	mg/dL	70-100	Hexokinase

Comment:

As per American Diabetes Guidelines, 2023

Fasting Glucose Values in mg/dL	Interpretation
70-100 mg/dL	Normal
100-125 mg/dL	Prediabetes
≥126 mg/dL	Diabetes
<70 mg/dL	Hypoglycemia

Note:

- The diagnosis of Diabetes requires a fasting plasma glucose of ≥ 126 mg/dL and/or a random / 2 hr post glucose value of ≥ 200 mg/dL on at least 2 occasions.
- Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.



Test Name	Result	Unit	Bio. Ref. Range	Method
HBA1C (GLYCATED HEMOGLOBIN) , WHOLE BLOOD EDTA				
HBA1C, GLYCATED HEMOGLOBIN	5.9	%		HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	123	mg/dL		Calculated

Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

REFERENCE GROUP	HBA1C %
NON DIABETIC	<5.7
PREDIABETES	5.7 – 6.4

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DEPARTMENT OF BIOCHEMISTRY

XPRT HEALTH SENIOR CITIZEN- FEMALE

DIABETES	≥ 6.5
DIABETICS	
EXCELLENT CONTROL	6 – 7
FAIR TO GOOD CONTROL	7 – 8
UNSATISFACTORY CONTROL	8 – 10
POOR CONTROL	>10

Note: Dietary preparation or fasting is not required.

1. HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.

2. Trends in HbA1C values is a better indicator of Glycemic control than a single test.

3. Low HbA1C in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.

4. Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.

5. In cases of Interference of Hemoglobin variants in HbA1C, alternative methods (Fructosamine) estimation is recommended for Glycemic Control

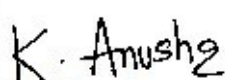
A: HbF >25%

B: Homozygous Hemoglobinopathy.

(Hb Electrophoresis is recommended method for detection of Hemoglobinopathy)



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SIN No:BI18279946

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DEPARTMENT OF BIOCHEMISTRY

XPRT HEALTH SENIOR CITIZEN- FEMALE

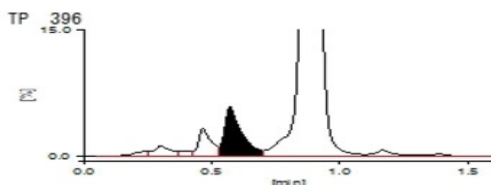
Chromatogram Report

HLC723G8 V5.28 1 2024-02-13 13:22:17
ID B118279946
Sample No. 02130079 SL 0001 - 04
Patient ID
Name
Comment

CALIB	Y = 1.1689X + 0.6048		
Name	%	Time	Area
A1A	0.4	0.23	5.00
A1B	1.0	0.30	12.01
F	0.3	0.39	4.06
LA1C+	1.9	0.47	23.57
SA1C	5.9	0.57	57.34
A0	92.1	0.87	1149.20
H-V0			
H-V1			
H-V2			

Total Area 1251.18

HbA1c 5.9 % IFCC 41 mmol/mol
HbA1 7.2 % HbF 0.3 %



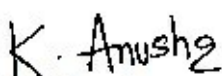
13-02-2024 13:22:18 APOLLO

APOLLO DIAGNOSTICS GLOBAL
BALANAGER

1 / 1



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DEPARTMENT OF BIOCHEMISTRY

XPERT HEALTH SENIOR CITIZEN- FEMALE



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DEPARTMENT OF BIOCHEMISTRY

XPRT HEALTH SENIOR CITIZEN- FEMALE

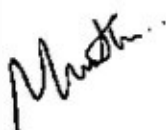
Test Name	Result	Unit	Bio. Ref. Range	Method
LIPID PROFILE , SERUM				
TOTAL CHOLESTEROL	213	mg/dL	<200	CHO-POD
TRIGLYCERIDES	133	mg/dL	<150	GPO-POD
HDL CHOLESTEROL	50	mg/dL	40-60	Enzymatic Immunoinhibition
NON-HDL CHOLESTEROL	163	mg/dL	<130	Calculated
LDL CHOLESTEROL	136.4	mg/dL	<100	Calculated
VLDL CHOLESTEROL	26.6	mg/dL	<30	Calculated
CHOL / HDL RATIO	4.26		0-4.97	Calculated

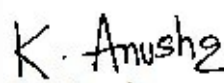
Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.

	Desirable	Borderline High	High	Very High
TOTAL CHOLESTEROL	< 200	200 - 239	≥ 240	
TRIGLYCERIDES	<150	150 - 199	200 - 499	≥ 500
LDL	Optimal < 100 Near Optimal 100-129	130 - 159	160 - 189	≥ 190
HDL	≥ 60			
NON-HDL CHOLESTEROL	Optimal <130; Above Optimal 130-159	160-189	190-219	>220

- Measurements in the same patient on different days can show physiological and analytical variations.
- NCEP ATP III identifies non-HDL cholesterol as a secondary target of therapy in persons with high triglycerides.
- Primary prevention algorithm now includes absolute risk estimation and lower LDL Cholesterol target levels to determine eligibility of drug therapy.
- Low HDL levels are associated with Coronary Heart Disease due to insufficient HDL being available to participate in reverse cholesterol transport, the process by which cholesterol is eliminated from peripheral tissues.
- As per NCEP guidelines, all adults above the age of 20 years should be screened for lipid status. Selective screening of children above the age of 2 years with a family history of premature cardiovascular disease or those with at least one parent with high total cholesterol is recommended.
- VLDL, LDL Cholesterol Non HDL Cholesterol, CHOL/HDL RATIO, LDL/HDL RATIO are calculated parameters when Triglycerides are below 350mg/dl. When Triglycerides are more than 350 mg/dl LDL cholesterol is a direct measurement.


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DEPARTMENT OF BIOCHEMISTRY

XPRT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
LIVER FUNCTION TEST (LFT) , SERUM				
BILIRUBIN, TOTAL	0.68	mg/dL	0.3–1.2	DPD
BILIRUBIN CONJUGATED (DIRECT)	0.11	mg/dL	<0.2	DPD
BILIRUBIN (INDIRECT)	0.57	mg/dL	0.0-1.1	Dual Wavelength
ALANINE AMINOTRANSFERASE (ALT/SGPT)	17	U/L	<35	IFCC
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	22.0	U/L	<35	IFCC
ALKALINE PHOSPHATASE	67.00	U/L	30-120	IFCC
PROTEIN, TOTAL	8.31	g/dL	6.6-8.3	Biuret
ALBUMIN	4.40	g/dL	3.5-5.2	BROMO CRESOL GREEN
GLOBULIN	3.91	g/dL	2.0-3.5	Calculated
A/G RATIO	1.13		0.9-2.0	Calculated

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin)

Common patterns seen:

1. Hepatocellular Injury:

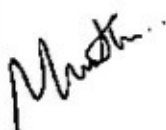
- AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries.
- ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI.
- Disproportionate increase in AST, ALT compared with ALP.
- Bilirubin may be elevated.
- AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1 In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilson's diseases, Cirrhosis, but the increase is usually not >2.

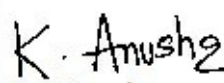
2. Cholestatic Pattern:

- ALP – Disproportionate increase in ALP compared with AST, ALT.
- Bilirubin may be elevated.
- ALP elevation also seen in pregnancy, impacted by age and sex.
- To establish the hepatic origin correlation with GGT helps. If GGT elevated indicates hepatic cause of increased ALP.

3. Synthetic function impairment:

- Albumin- Liver disease reduces albumin levels.
- Correlation with PT (Prothrombin Time) helps.


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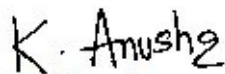
DEPARTMENT OF BIOCHEMISTRY

XPERT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) , SERUM				
CREATININE	0.90	mg/dL	0.66 - 1.09	Modified Jaffe, Kinetic
UREA	24.60	mg/dL	17-43	GLDH, Kinetic Assay
BLOOD UREA NITROGEN	11.5	mg/dL	8.0 - 23.0	Calculated
URIC ACID	8.45	mg/dL	2.6-6.0	Uricase PAP
CALCIUM	9.76	mg/dL	8.8-10.6	Arsenazo III
PHOSPHORUS, INORGANIC	3.34	mg/dL	2.5-4.5	Phosphomolybdate Complex
SODIUM	136	mmol/L	136-146	ISE (Indirect)
POTASSIUM	4.4	mmol/L	3.5-5.1	ISE (Indirect)
CHLORIDE	101	mmol/L	101-109	ISE (Indirect)



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DEPARTMENT OF BIOCHEMISTRY

XPERT HEALTH SENIOR CITIZEN- FEMALE

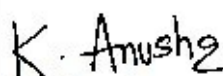
Test Name	Result	Unit	Bio. Ref. Range	Method
RHEUMATOID FACTOR , SERUM	3.10	IU/mL	<14	Latex Particle Immunoturbidimetric

Comment:

Rheumatoid factor (RF) consists of autoantibodies of immunoglobulin isotypes IgM, IgA, IgG, and IgE. Most patients with rheumatoid arthritis and Sjogren's syndrome have elevated levels of RF. RF may also be elevated in scleroderma, dermatomyositis, Waldenstrom's disease, sarcoidosis, and systemic lupus erythematosus. There are also instances of elevated RF without apparent disease or definable clinical disorders.




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DEPARTMENT OF BIOCHEMISTRY

XPERT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
IRON STUDIES (IRON + TIBC) , SERUM				
IRON	59.2	µg/dL	60-180	TPTZ AND NITROSO-PSAP
TOTAL IRON BINDING CAPACITY (TIBC)	388	µg/dL	261-462	Calculated
UNSATURATED IRON BINDING CAPACITY (UIBC)	328.50	IU/mL	155-355	NITROSO-PSAP
% OF SATURATION	15.26	%	14-50	Calculated

Comment:

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Diurnal variation is seen in serum iron levels—normal values in midmorning, low values in midafternoon, very low values (approximately 10 µg/dL) near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

% saturation represents the amount of iron-binding sites that are occupied. Iron saturation is a better index of iron stores than serum iron alone. % saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).



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DEPARTMENT OF IMMUNOLOGY

XPRT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
THYROID PROFILE TOTAL (T3, T4, TSH) , SERUM				
TRI-iodothyronine (T3, TOTAL)	1.24	ng/mL	0.87-1.78	CLIA
THYROXINE (T4, TOTAL)	12.23	µg/dL	5.48-14.28	CLIA
THYROID STIMULATING HORMONE (TSH)	6.047	µIU/mL	0.38-5.33	CLIA

Comment:

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

1. TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH activates production of T3 (Triiodothyronine) and its prohormone T4 (Thyroxine). Increased blood level of T3 and T4 inhibit production of TSH.
2. TSH is elevated in primary hypothyroidism and will be low in primary hyperthyroidism. Elevated or low TSH in the context of normal free thyroxine is often referred to as sub-clinical hypo- or hyperthyroidism respectively.
3. Both T4 & T3 provides limited clinical information as both are highly bound to proteins in circulation and reflects mostly inactive hormone. Only a very small fraction of circulating hormone is free and biologically active.
4. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, medication & circulating antibodies.

TSH	T3	T4	FT4	Conditions
High	Low	Low	Low	Primary Hypothyroidism, Post Thyroidectomy, Chronic Autoimmune Thyroiditis
High	N	N	N	Subclinical Hypothyroidism, Autoimmune Thyroiditis, Insufficient Hormone Replacement Therapy.
N/Low	Low	Low	Low	Secondary and Tertiary Hypothyroidism
Low	High	High	High	Primary Hyperthyroidism, Goitre, Thyroiditis, Drug effects, Early Pregnancy
Low	N	N	N	Subclinical Hyperthyroidism
Low	Low	Low	Low	Central Hypothyroidism, Treatment with Hyperthyroidism
Low	N	High	High	Thyroiditis, Interfering Antibodies
N/Low	High	N	N	T3 Thyrotoxicosis, Non thyroidal causes
High	High	High	High	Pituitary Adenoma; TSHoma/Thyrotropinoma


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SIN No:IM06960251

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DEPARTMENT OF IMMUNOLOGY

XPERT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
VITAMIN D (25 - OH VITAMIN D) , SERUM	26.9	ng/mL	30 -100	CLIA

Comment:

BIOLOGICAL REFERENCE RANGES

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL)
DEFICIENCY	<10
INSUFFICIENCY	10 – 30
SUFFICIENCY	30 – 100
TOXICITY	>100

The biological function of Vitamin D is to maintain normal levels of calcium and phosphorus absorption. 25-Hydroxy vitamin D is the storage form of vitamin D. Vitamin D assists in maintaining bone health by facilitating calcium absorption. Vitamin D deficiency can also cause osteomalacia, which frequently affects elderly patients.

Vitamin D Total levels are composed of two components namely 25-Hydroxy Vitamin D2 and 25-Hydroxy Vitamin D3 both of which are converted into active forms. Vitamin D2 level corresponds with the exogenous dietary intake of Vitamin D rich foods as well as supplements. Vitamin D3 level corresponds with endogenous production as well as exogenous diet and supplements.

Vitamin D from sunshine on the skin or from dietary intake is converted predominantly by the liver into 25-hydroxy vitamin D, which has a long half-life and is stored in the adipose tissue. The metabolically active form of vitamin D, 1,25-di-hydroxy vitamin D, which has a short life, is then synthesized in the kidney as needed from circulating 25-hydroxy vitamin D. The reference interval of greater than 30 ng/mL is a target value established by the Endocrine Society.

Decreased Levels:

Inadequate exposure to sunlight.

Dietary deficiency.

Vitamin D malabsorption.

Severe Hepatocellular disease.

Drugs like Anticonvulsants.

Nephrotic syndrome.

Increased levels:

Vitamin D intoxication.



Dr. RAJESH BATTINA
PhD.(Biochemistry)
Consultant Biochemist

SIN No:IM06960251

This test has been performed at Apollo Health & Lifestyle Ltd, Global Reference Laboratory,Hyderabad



Patient Name	: Mrs.NITA SINGH	Collected	: 13/Feb/2024 09:58AM
Age/Gender	: 58 Y 9 M 10 D /F	Received	: 13/Feb/2024 01:20PM
UHID/MR No	: DPPU.0000002063	Reported	: 13/Feb/2024 02:57PM
Visit ID	: DPPUOPV4758	Status	: Final Report
Ref Doctor	: Dr.SELF	Client Name	: PCC UPPERPALLY
IP/OP NO	:	Patient location	: ,Hyderabad

DEPARTMENT OF IMMUNOLOGY

XPRT HEALTH SENIOR CITIZEN- FEMALE




Dr.RAJESH BATTINA
PhD.(Biochemistry)
Consultant Biochemist

SIN No:IM06960251

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Patient Name : Mrs.NITA SINGH
Age/Gender : 58 Y 9 M 10 D /F
UHID/MR No : DPPU.0000002063
Visit ID : DPPUOPV4758
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 13/Feb/2024 09:58AM
Received : 13/Feb/2024 01:20PM
Reported : 13/Feb/2024 03:25PM
Status : Final Report
Client Name : PCC UPPERPALLY
Patient location : ,Hyderabad

DEPARTMENT OF IMMUNOLOGY

XPRT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
VITAMIN B12 , SERUM	471	pg/mL	107.2-653.3	CLIA

Comment:

- Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes.
- The most common cause of deficiency is malabsorption either due to atrophy of gastric mucosa or diseases of terminal ileum. Patients taking vitamin B12 supplementation may have misleading results.
- A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12 .
- The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.
- Increased levels can be seen in Chronic renal failure, Congestive heart failure, Leukemias, Polycythemia vera, Liver disease etc.



K. Anusha
Dr.K.Anusha
M.B.B.S,M.D(Biochemistry)
Consultant Biochemist

SIN No:IM06960251

This test has been performed at Apollo Health & Lifestyle Ltd, Global Reference Laboratory,Hyderabad



Patient Name : Mrs.NITA SINGH
Age/Gender : 58 Y 9 M 10 D /F
UHID/MR No : DPPU.0000002063
Visit ID : DPPUOPV4758
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 13/Feb/2024 09:58AM
Received : 13/Feb/2024 01:20PM
Reported : 13/Feb/2024 02:04PM
Status : Final Report
Client Name : PCC UPPERPALLY
Patient location : ,Hyderabad

DEPARTMENT OF IMMUNOLOGY

XPERT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
CA -125 , SERUM	3.6	U/mL	0-35	CLIA

Comment:

1. This test is not suggested for screening asymptomatic women
2. CA 125 is elevated in over 90% of patients with advanced ovarian cancer. Many other advanced malignancies can also secrete the CA125 antigen such as breast, pancreas, colon, lung, or liver carcinoma. CA125 antigen has also been reported to be elevated in non-malignant conditions
3. Sequential determinations are more useful than a single test, because levels in benign disease do not show significant change but progressive rise occurs in malignant disease
4. Measurements may also be used to monitor response to chemotherapy. Rising level of CA-125 during chemotherapy is associated with tumor progression, and fall to normal is associated with response. It remains elevated in stable or progressive serous carcinoma of the ovary
5. It should be noted that 0.6% of normal women older than 50 years of age have increased levels of CA-125.
6. Normal concentration of CA-125 does not exclude tumor



Dr.RAJESH BATTINA
PhD.(Biochemistry)
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SIN No:IM06960251

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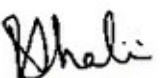
Patient Name	: Mrs.NITA SINGH	Collected	: 13/Feb/2024 09:58AM
Age/Gender	: 58 Y 9 M 10 D /F	Received	: 13/Feb/2024 01:18PM
UHID/MR No	: DPPU.0000002063	Reported	: 13/Feb/2024 04:24PM
Visit ID	: DPPUOPV4758	Status	: Final Report
Ref Doctor	: Dr.SELF	Client Name	: PCC UPPERPALLY
IP/OP NO	:	Patient location	: ,Hyderabad

DEPARTMENT OF CLINICAL PATHOLOGY

XPRT HEALTH SENIOR CITIZEN- FEMALE

Test Name	Result	Unit	Bio. Ref. Range	Method
COMPLETE URINE EXAMINATION (CUE) , URINE				
PHYSICAL EXAMINATION				
COLOUR	PALE YELLOW		PALE YELLOW	Visual
TRANSPARENCY	CLEAR		CLEAR	Visual
pH	6.0		5-7.5	Bromothymol Blue
SP. GRAVITY	1.020		1.002-1.030	Bromothymol Blue
BIOCHEMICAL EXAMINATION				
URINE PROTEIN	NEGATIVE		NEGATIVE	PROTEIN ERROR OF INDICATOR
GLUCOSE	NEGATIVE		NEGATIVE	GOD - POD
URINE BILIRUBIN	NEGATIVE		NEGATIVE	AZO COUPLING
URINE KETONES (RANDOM)	NEGATIVE		NEGATIVE	SODIUM NITRO PRUSSIDE
UROBILINOGEN	NORMAL		NORMAL	EHRlich
BLOOD	NEGATIVE		NEGATIVE	Peroxidase
NITRITE	NEGATIVE		NEGATIVE	Diazotization
LEUCOCYTE ESTERASE	NEGATIVE		NEGATIVE	PYRROLE HYDROLYSIS
CENTRIFUGED SEDIMENT WET MOUNT AND MICROSCOPY				
PUS CELLS	3-4	/hpf	0-5	Microscopy
EPITHELIAL CELLS	1-2	/hpf	<10	MICROSCOPY
RBC	NIL	/hpf	0-2	MICROSCOPY
CASTS	NIL		0-2 Hyaline Cast	MICROSCOPY
CRYSTALS	ABSENT		ABSENT	MICROSCOPY

*** End Of Report ***


Dr.R.SHALINI
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Consultant Pathologist

SIN No:C02600454

This test has been performed at Apollo Health & Lifestyle Ltd, Global Reference Laboratory,Hyderabad

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Apollo Health and Lifestyle Limited

(CIN - U85110TG2000PLC115819)

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